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MED 101: Introduction to Medicine - History Taking, Ethics and Professionalism

The human and professional foundations of medical training in Nigeria

FACULTY PRIMER

Faculty Primer

Faculty of Medicine
Community Medicine & Primary Health Care | 100L

Written for students, by students.

Practical notes, worked examples, practice questions and a revision plan.

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Independent study support | Not an official UNIZIK publication

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Why this pack exists

Medical school begins with learning how to listen. This pack introduces patient history taking, communication, confidentiality, consent and the Nigerian medical code of conduct, along with a practical study routine for a course load that punishes passive reading.

This is a student-written study companion, not a reproduced lecture handout or a leaked examination paper. Use it to organise the topics you are taught, practise the kind of thinking your course rewards, and walk into class with better questions.

What you will be able to do

- Take a structured patient history using a clear sequence.
- Apply the four principles of biomedical ethics to common clinical scenarios.
- Explain confidentiality, informed consent and professional boundaries.
- Build a sustainable study routine for high-volume medical content.

How to use this pack

1. Skim the whole pack once in 20 minutes so you know what is inside.
2. Work through one section at a time with your lecture note and recommended text beside you.
3. Attempt the practice questions before reading the answers. Mark what you missed.
4. Use the final checklist to plan your revision week, one tick per study session.

> **Free to download, always**
> This pack is free. There is no fee, no subscription and no payment step before download, and the full PDF is not watermarked or locked. Share the link with a coursemate instead of forwarding the file.

> **Confirm your official course outline**
> Course codes, credit units and outlines can differ by department and session. Treat this pack as supplementary support and confirm the current outline with your lecturer, course rep or departmental handbook before an assessment.

Structure of a history

A complete history is a method, not a conversation you improvise. Practice the sequence until it becomes habitual.

1. Bio-data and presenting complaint, in the patient's own words plus duration.
2. History of presenting complaint using the mnemonic for the relevant symptoms.
3. Past medical and surgical history, drug history, allergies.
4. Family, social and occupational history.
5. Review of systems.

6. Summarise back to the patient and confirm that you understood correctly.

> **Open then closed**
> Begin with open questions such as 'tell me what brought you in', then narrow with closed questions. Jumping straight to closed questions produces incomplete histories.

Ethics that will actually be tested

Ethical reasoning is a subject with principles, not a personality trait.

- Autonomy: respect the patient's right to decide, supported by adequate information.
- Beneficence: act in the patient's best interest.
- Non-maleficence: avoid avoidable harm.
- Justice: treat similar cases similarly and use scarce resources fairly.
- Confidentiality: information is shared only with those involved in care or as legally required.
- Informed consent requires capacity, adequate disclosure, understanding and voluntariness.

Professional behaviour and self-management

Studies show that unprofessional behaviour in training predicts later problems. Start the habits now.

- Arrive early, dressed appropriately, with the tools you need.
- Introduce yourself to patients and ask permission before examination.
- Do not discuss patients in corridors, lifts or on WhatsApp.
- Document honestly and promptly. A wrong note left uncorrected can harm a patient.
- Study with spaced repetition and active recall rather than rereading slides.
- Protect sleep, exercise and relationships - burnout damages clinical judgement.

> **The code of conduct**
> Read the current Nigerian Medical and Dental Council code rather than relying on hearsay. Know the sections on confidentiality, advertising and relationships with colleagues.

The clinical method

Medicine begins with a structured approach: history, examination, differential diagnosis, investigation, management. Each stage narrows the possibilities before the next, and most diagnoses are made from the history alone.

A differential is a working list ranked by likelihood and by danger. Rank by what is common, but never omit what is fatal - a rare diagnosis that kills matters more than a common one that resolves. This is the principle behind ruling out the worst first.